

Adult Nursing Question Bank

Built from ordered ZIP folder. Questions that need review are marked.

Chapter 1 - Urinary System Disorders

TRUE / FALSE

1. A urinary tract infection (UTI) can occur in the kidneys, ureters, bladder, or urethra.

Answer: T

2. Women get UTIs less frequently than men because their urethra is longer.

Answer: F

3. Elderly people in nursing homes are at higher risk for UTIs.

Answer: T

4. Pregnancy is NOT a risk factor for developing a UTI.

Answer: F

5. Immobility is a risk factor for urinary tract infection.

Answer: T

6. Hematuria (blood in the urine) is a symptom of UTI.

Answer: T

7. Nocturia refers to the absence of urination at night.

Answer: F

8. A urine culture is performed to identify the type of bacteria and choose the appropriate antibiotic.

Answer: T

9. A mild case of cystitis always requires antibiotic treatment.

Answer: F

10. Chronic or recurrent UTI may require antibiotic therapy for up to 6 months to 2 years.

Answer: T

11. Patients should wipe from back to front after toileting to prevent UTIs.

Answer: F

12. Urinating after sexual intercourse helps prevent UTIs.

Answer: T

13. Alcohol and caffeine are recommended fluids for patients with UTI.

Answer: F

14. Renal failure is defined as total or nearly total loss of kidney function.

Answer: T

15. There is only one type of renal failure.

Answer: F

16. Diabetic nephropathy is the most common/associated/chronic renal failure.

Answer: T

17. Hypertension nephrosclerosis is the leading cause of chronic renal failure.

Answer: F

18. Neurogenic bladder is caused by impaired nerve fibers from the bladder to the spinal cord.

Answer: T

19. Bladder outlet obstruction due to an enlarged prostate gland is a post-renal cause of CRF.

CHECKED FROM IMAGE

Answer: T

20. Neurogenic bladder is caused by impaired nerve fibers from the bladder to the spinal cord.

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Answer: T

21. Poor cardiac function is a renal cause of chronic renal failure.

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Answer: F

22. Kussmaul breathing in CRF patients is caused by metabolic alkalosis.

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Answer: F

23. Pruritus (itching) in CRF patients is due to accumulation of urea frost on the skin.

CHECKED FROM IMAGE

Answer: T

24. Salt and water retention in CRF can lead to congestive heart failure and hypertension.

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Answer: T

25. Hemodialysis works by circulating blood through a semi-permeable membrane (dialyzer) to remove toxins.

CHECKED FROM IMAGE

Answer: T

26. An AV fistula involves the surgical joining of an artery and vein under the skin.

CHECKED FROM IMAGE

Answer: T

27. Kidney stones in both ureters are a pre-renal cause of CRF.

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Answer: F

28. Renal biopsy is one of the laboratory/diagnostic studies used to evaluate CRF.

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Answer: T

29. Osteomalacia and renal osteodystrophy are skeletal complications of chronic renal failure.

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Answer: T

30. Dialysis is only indicated when a kidney transplant is not possible.

CHECKED FROM IMAGE

Answer: F

MCQ

31. A UTI is best defined as:

- A. Inflammation of the kidney parenchyma only
- B. An infection anywhere along the urinary tract including kidneys, ureters, bladder, or urethra
- C. A bacterial infection limited to the urethra
- D. A viral infection of the bladder only

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Answer: B

32. Which anatomical reason makes women more prone to UTIs than men?

- A. Women have a wider urethra
- B. Women have a longer urethra closer to the vagina
- C. Women have a shorter urethra closer to the anus
- D. Women produce less urine

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Answer: C

33. Which of the following is NOT listed as a risk factor for UTI?

- A. Pregnancy
- B. Kidney stones
- C. Regular physical exercise
- D. Catheterization

CHECKED FROM IMAGE

Answer: C

34. Cloudy urine and foul odor are symptoms of:

- A. Diabetes mellitus
- B. Urinary tract infection
- C. Chronic renal failure
- D. Renal calculi

CHECKED FROM IMAGE

Answer: B

35. The diagnostic test that identifies the specific bacteria causing a UTI and guides antibiotic choice is:

- A. Complete blood count
- B. Urinalysis
- C. Urine culture
- D. Renal biopsy

Answer: C

36. Urinalysis in a patient with UTI typically reveals:

- A. Glucose and ketones
- B. White blood cells or red blood cells
- C. Elevated protein only
- D. Normal findings

Answer: B

37. A patient with chronic or recurrent UTI may require antibiotic therapy for:

- A. 3 to 5 days
- B. 1 to 3 weeks
- C. 1 to 3 months
- D. 6 months to 2 years

Answer: D

38. Which instruction is MOST important to prevent UTIs in a female patient?

- A. Wipe from back to front
- B. Avoid drinking water
- C. Wipe from front to back
- D. Use synthetic undergarments

Answer: C

39. Which of the following fluids should be AVOIDED by a UTI patient?

- A. Water
- B. Cranberry juice
- C. Herbal tea
- D. Alcohol

Answer: D

40. Urinating after sexual intercourse is recommended because it helps:

- A. Prevent pregnancy
- B. Flush bacteria from the urethra
- C. Reduce bladder spasms
- D. Decrease urine production

Answer: B

41. Surgery for UTI is recommended when:

- A. The patient has recurrent UTIs
- B. The patient is elderly
- C. An anatomical abnormality is present causing the infection
- D. Antibiotic therapy has lasted 1 week

Answer: C

42. Wearing cloth (cotton) undergarments is recommended to prevent UTIs because:

- A. Cotton is more absorbent of urine
- B. Cotton allows better air circulation, reducing moisture and bacterial growth
- C. Synthetic materials are more comfortable
- D. Cotton kills bacteria directly

Answer: B

43. Renal failure is defined as:

- A. Temporary reduction in urine output
- B. Inflammation of the kidney
- C. Total or nearly total loss of kidney function
- D. Obstruction of the ureter

Answer: C

44. How many types of renal failure are described?

- A. One
- B. Two
- C. Three
- D. Four

Answer: B

45. Which classification includes poor cardiac function as a cause of CRF?

- A. Renal causes
- B. Post-renal causes
- C. Pre-renal causes
- D. Intrinsic renal causes

Answer: C

46. Neurogenic bladder causing CRF is classified as:

- A. Pre-renal cause
- B. Post-renal cause
- C. Intrinsic renal cause
- D. Infectious cause

Answer: B

47. The MOST common cause of chronic renal failure is:

- A. Hypertension Nephrosclerosis
- B. Polycystic kidney disease
- C. Diabetic nephropathy
- D. Chronic glomerulonephritis

Answer: C

48. The SECOND leading cause of chronic renal failure is:

- A. Polycystic kidney disease
- B. Diabetic nephropathy
- C. Hypertension Nephrosclerosis
- D. Chronic pyelonephritis

Answer: C

49. Polycystic kidney disease is characterized by:

- A. Inflammation of the glomeruli
- B. Multiple cysts in the kidneys
- C. Scarring of the renal cortex
- D. Enlarged renal pelvis

Answer: B

50. Which gastrointestinal symptom is associated with chronic renal failure?

- A. Melena and constipation
- B. Anorexia, nausea, vomiting, and diarrhea
- C. Jaundice and hepatomegaly
- D. Dysphagia and heartburn

Answer: B

51. Rapid deep breathing (Kussmaul breathing) in CRF patients is caused by:

- A. Pulmonary edema
- B. Metabolic alkalosis
- C. Metabolic acidosis
- D. Respiratory infection

Answer: C

52. Pruritus (generalised itching) in CRF patients is caused by:

- A. Liver failure
- B. Allergic reaction to medications
- C. Accumulation of urea frost on the skin
- D. Calcium deposits in the skin

Answer: C

53. Which skin change is associated with CRF?

- A. Pale white skin
- B. Red flushed skin
- C. Yellow or brown skin discolouration with increased pigmentation
- D. Blue cyanotic skin

Answer: C

54. Salt and water retention in CRF leads to all of the following EXCEPT:

- A. Edema
- B. Congestive heart failure
- C. Hypertension
- D. Hypotension

Answer: D

55. Neurological manifestations of CRF include all of the following EXCEPT:

- A. Confusion and drowsiness
- B. Muscle cramps and seizures
- C. Coma
- D. Improved mental alertness

Answer: D

56. Which laboratory panel is used in the diagnostic evaluation of CRF?

- A. Liver function tests only
- B. Full electrolyte panel including calcium, phosphate, uric acid, magnesium, and albumin
- C. Thyroid function tests
- D. Coagulation studies only

Answer: B

57. Which radiographic study evaluates for obstruction, stones, tumour, and kidney size in CRF?

- A. MRI
- B. Angiography
- C. Renal ultrasound
- D. Chest X-ray

Answer: C

58. The goal of conservative management in CRF is:

- A. Complete cure of the disease
- B. Retaining kidney function as much as possible
- C. Immediate preparation for transplant
- D. Replacing kidney function entirely

Answer: B

59. Which medication may be used to treat anemia in CRF patients?

- A. Insulin injections
- B. Iron and erythropoietin injections
- C. Calcium supplements only
- D. Diuretics

Answer: B

60. Dialysis is a process that:

- A. Cures chronic renal failure permanently
- B. Replaces only hormone functions of the kidney
- C. Removes fluid, waste products, and toxic substances from blood when kidneys cannot work
- D. Increases urine production in failing kidneys

Answer: C

61. In hemodialysis, blood is circulated through:

- A. The peritoneal cavity
- B. A semi-permeable membrane (dialyzer)
- C. A synthetic heart valve
- D. A venous reservoir only

Answer: B

62. Vascular access for hemodialysis can be achieved through:

- A. A central venous catheter only
- B. An AV fistula or AV graft
- C. A peripheral IV line
- D. A Foley catheter

Answer: B

63. An AV fistula is created by:

- A. Inserting a synthetic tube between artery and vein
- B. Surgically joining an artery and vein under the skin
- C. Cannulating the femoral artery
- D. Placing a catheter in the subclavian vein

Answer: B

64. The two replacement therapy options for lost kidney function are:

- A. Hemodialysis and peritoneal dialysis only
- B. Transplant and dialysis
- C. Medications and dietary changes
- D. Surgery and radiation

Answer: B

65. Atherosclerosis of the renal arteries causes CRF by:

- A. Blocking urine outflow from the kidney
- B. Directly destroying nephrons
- C. Inducing ischaemic nephropathy through reduced blood flow
- D. Causing polycystic changes

Answer: C

66. Bone disease in CRF (renal osteodystrophy) is caused by:

- A. Excessive calcium absorption
- B. Impaired calcium and phosphate metabolism due to kidney failure
- C. Vitamin B12 deficiency
- D. Steroid medication use only

Answer: B

67. Which of the following correctly describes pressure in the lower pelvis as a symptom?

- A. It is specific to chronic renal failure
- B. It is a symptom of UTI
- C. It only occurs in men with prostate disease
- D. It is a neurological symptom of CRF

Answer: B

68. Bowel incontinence is a risk factor for UTI because:

- A. It reduces fluid intake
- B. Faecal contamination increases bacterial access to the urethra
- C. It causes bladder stones
- D. It affects kidney function directly

Answer: B

69. Frequent and urgent need to urinate as a symptom of UTI is also called:

- A. Oliguria
- B. Dysuria
- C. Urinary urgency/frequency
- D. Anuria

Answer: C

70. Which of the following is a pre-renal cause of chronic renal failure?

- A. Polycystic kidney disease
- B. Bladder stone causing outlet obstruction
- C. Chronic liver failure reducing renal perfusion
- D. Kidney stones in both ureters

Answer: C

71. Keeping the genital area clean is a preventive measure for:

- A. Chronic renal failure
- B. Urinary tract infection
- C. Kidney stones
- D. Neurogenic bladder

Answer: B

72. Which finding on urinalysis indicates possible UTI?

- A. Presence of glucose
- B. Presence of white blood cells (WBCs)
- C. Normal specific gravity
- D. Absence of protein

Answer: B

73. Decreased fluid intake is a risk factor for UTI because:

- A. It increases urine concentration, killing bacteria
- B. It causes urinary stasis, allowing bacteria to multiply
- C. It reduces kidney function directly
- D. It increases bladder capacity

Answer: B

74. A patient with CRF has fatigue, headache, and a general feeling of illness. This is called:

- A. Uremia
- B. Azotemia
- C. General ill feeling (uremic syndrome)
- D. Nephrotic syndrome

Answer: C

75. Angiography or CT scan in CRF patients is used to evaluate:

- A. Bladder capacity
- B. Renal artery stenosis
- C. Urinary flow rate
- D. Electrolyte levels

Answer: B

76. In CRF management, which associated diseases must be controlled first?

- A. Diabetes only
- B. Hypertension, congestive heart failure, UTIs, and kidney stones
- C. Anemia and osteoporosis only
- D. Neurological disorders

Answer: B

77. Decreased sensation in hands, feet, or other areas in CRF is a manifestation of:

- A. Cardiovascular complications
- B. Neurological manifestations (peripheral neuropathy)
- C. Skin changes
- D. Gastrointestinal complications

Answer: B

78. The purpose of erythropoietin injection in CRF management is to:

- A. Lower blood pressure
- B. Stimulate red blood cell production and control anemia
- C. Reduce potassium levels
- D. Treat metabolic acidosis

Answer: B

79. Which preventive measure helps reduce bacterial growth by maintaining a dry perineal environment?

- A. Using synthetic underwear
- B. Wearing tight-fitting clothing
- C. Wearing cloth (cotton) undergarments
- D. Using scented soaps

Answer: C

80. Coma in a patient with CRF is classified as a:

- A. Cardiovascular complication
- B. Neurological manifestation
- C. Dermatological change
- D. Gastrointestinal symptom

Answer: B

81. Which of the following correctly states the relationship between diabetes and CRF?

- A. Diabetes never causes kidney disease
- B. Diabetic nephropathy is the most common cause of CRF
- C. Diabetes causes only pre-renal failure
- D. CRF causes diabetes mellitus

Answer: B

82. Ascites in CRF patients results from:

- A. Liver cirrhosis only
- B. Salt and water retention causing fluid accumulation in the peritoneum
- C. Excessive protein intake
- D. Calcium deposits in the abdomen

Answer: B

83. Nail abnormalities in CRF patients are classified as:

- A. Neurological manifestations
- B. Gastrointestinal symptoms
- C. Skin and nail changes
- D. Cardiovascular complications

Answer: C

84. The purpose of renal ultrasound in CRF diagnosis is to:

- A. Measure glomerular filtration rate
- B. Evaluate for obstruction, stones, tumour, kidney size, and chronic change
- C. Determine urine culture results
- D. Assess renal blood flow only

Answer: B

85. Which of the following is a correct prevention strategy for UTIs?

- A. Drink plenty of fluids to flush bacteria
- B. Reduce fluid intake to rest the bladder
- C. Use synthetic undergarments for better protection
- D. Urinate less frequently to retain bladder capacity

Answer: A

86. Hiccups in a patient with CRF are classified as:

- A. A respiratory symptom
- B. A gastrointestinal manifestation of CRF
- C. A neurological sign
- D. A cardiovascular complication

Answer: B

87. Pain or burning with urination is medically termed:

- A. Nocturia
- B. Hematuria
- C. Dysuria
- D. Proteinuria

Answer: C

88. The process by which blood is cleaned in hemodialysis uses the principle of:

- A. Osmosis only
- B. Diffusion across a semi-permeable membrane
- C. Filtration through the renal cortex
- D. Active transport of solutes

Answer: B

89. Which of the following conditions is associated with an increased risk of UTI due to urinary stasis?

- A. Regular aerobic exercise
- B. Immobility and decreased fluid intake
- C. High-protein diet
- D. Use of oral contraceptives

Answer: B

90. Dietary intervention is part of which aspect of CRF management?

- A. Replacement therapy
- B. Surgical management
- C. Conservative (general) management
- D. Pharmacological management only

Answer: C

MATCHING

1. UTI a. Swelling caused by fluid retention b

2. Cystitis b. Infection in any part of urinary tract c

3. Hematuria c. Inflammation of the bladder d

4. Nocturia d. Presence of blood in urine e

5. Edema e. Frequent urination at night a

6. Renal failure f. Loss of kidney function f

7. Hemodialysis g. Artificial cleansing of blood g

8. Anuria h. Absence of urine output h

9. Oliguria i. Decreased urine output i

10. Polyuria j. Excessive urine output j

11. Pruritus k. Itching sensation k

12. Dysuria l. Painful urination l

13. Dyspnea m. Difficulty breathing m

14. Fatigue n. Extreme tiredness n

15. Nausea o. Feeling of wanting to vomit o

ESSAY

91. Which symptom in CRF results from the kidney's inability to excrete hydrogen

92. Define urinary tract infection (UTI) and describe its causes, risk factors, and

93. Outline the nursing education that should be provided to a patient to prevent

94. Classify the causes of chronic renal failure into pre-renal, renal, and post-

- 95.** Describe the signs and symptoms of chronic renal failure, organising them by
- 96.** Describe the conservative management of chronic renal failure and explain the
- 97.** Define hemodialysis and describe how it is performed, including vascular
- 98.** Compare and contrast acute renal failure and chronic renal failure.
- 99.** What are the dietary restrictions recommended for patients with chronic renal
- 100.** Write a comprehensive nursing care plan for a patient with chronic renal

Chapter 2 - Respiratory Tract Disorders

TRUE / FALSE

1. Pneumonia is inflammation of the airway and alveoli caused by infectious agents.

Answer: T

2. Inhalation pneumonia can be caused by gastric content.

Answer: T

3. Smoking is not considered a predisposing factor for pneumonia.

Answer: F

4. A depressed cough reflex increases the risk of pneumonia.

Answer: T

5. Sudden onset and shaking chills are signs of pneumonia.

Answer: T

6. Chest pain in pneumonia is usually relieved by coughing.

Answer: F

7. Tachypnea in pneumonia may be accompanied by nasal flaring.

Answer: T

8. Sputum culture is used as a diagnostic tool for pneumonia.

Answer: T

9. Frequent turning of bedridden patients helps prevent pneumonia.

Answer: T

10. Isolation of the patient is never necessary during the acute stage of pneumonia.

Answer: F

11. An upright position helps obtain greater lung expansion in pneumonia patients.

Answer: T

12. Warm fluid intake should be discouraged for pneumonia patients.

Answer: F

13. Postural drainage is a nursing management technique for pneumonia.

Answer: T

14. Hypotension and shock are potential complications of pneumonia.

Answer: T

15. Atelectasis in pneumonia is caused by accumulated secretions.

Answer: T

16. Pulmonary TB is a communicable bacterial infection.

Answer: T

17. TB is caused by the Mycobacterium tuberculosis bacteria.

Answer: T

18. TB can only affect the lungs and no other organs.

Answer: F

19. Primary stage TB infection is usually asymptomatic.

Answer: T

20. Drug-resistant bacteria can emerge from incomplete TB treatment.

Answer: T

21. Close contact with an active TB patient is a risk factor.

Answer: T

22. Children under age 15 are not at risk for TB.

Answer: F

23. Night sweats and weight loss are systemic symptoms of TB.

Answer: T

24. Hemoptysis means coughing up blood.

Answer: T

25. TB is spread through aerosol droplets from coughing or sneezing.

Answer: T

26. Latent TB patients can transmit the disease to others.

Answer: F

27. A tuberculin skin test is used in TB diagnosis.

Answer: T

28. Rifampicin and Isoniazid are common antibiotics for TB.

Answer: T

29. There is a reliable TB vaccine available for all adults.

Answer: F

30. Hospitalised TB patients should be kept in negative pressure rooms.

Answer: T

31. COPD airflow limitation is fully reversible.

Answer: F

32. Emphysema and chronic bronchitis are types of COPD.

Answer: T

33. Genetic deficiency of alpha antitrypsin is a risk factor for COPD.

Answer: T

34. Tobacco smoke is a primary risk factor for COPD.

Answer: T

35. Tachypnea is a clinical manifestation of COPD.

Answer: T

36. In COPD, breathing out takes a shorter time than breathing in.

Answer: F

37. Pursed-lip breathing is a sign/technique used by COPD patients.

Answer: T

38. Finger clubbing can be a sign of emphysema.

Answer: T

39. Quitting smoking is a medical management strategy for COPD.

Answer: T

40. Bronchodilators are used to dilate the airways.

Answer: T

41. Patients with COPD should avoid dust-free environments.

Answer: F

42. High fluid intake (2–3 litres) is recommended for COPD patients.

Answer: T

43. Dairy products should be avoided if they increase sputum production.

Answer: T

44. Tachycardia is a potential adverse effect of bronchodilators.

Answer: T

45. Right-sided heart failure is a complication of COPD.

Answer: T

MCQ

46. Which type of pneumonia is caused by inhaling gastric content?

- A. Viral
- B. Fungal
- C. Inhalation
- D. Bacterial

CHECKED FROM IMAGE

Answer: C

47. Which of the following is a predisposing factor for pneumonia?

- A. Exercise
- B. Immunosuppression
- C. High protein diet
- D. Youth

CHECKED FROM IMAGE

Answer: B

48. A common sign of pneumonia is sputum that is:

- A. Clear
- B. Frothy
- C. Purulent
- D. Absent

CHECKED FROM IMAGE

Answer: C

49. Diagnostic tests for pneumonia include:

- A. MRI
- B. Chest X-ray
- C. EEG
- D. Bone scan

CHECKED FROM IMAGE

Answer: B

50. To prevent aspiration in unconscious patients, the nurse should:

- A. Give oral fluids
- B. Suction secretions
- C. Keep them flat
- D. Avoid turning

CHECKED FROM IMAGE

Answer: B

51. What is the priority nursing action for a pneumonia patient with inadequate gas exchange?

- A. Give a bath
- B. Oxygen therapy
- C. Exercise
- D. Solid food

CHECKED FROM IMAGE

Answer: B

52. The best position for a patient with pneumonia to expand lungs is:

- A. Upright
- B. Supine
- C. Prone
- D. Side-lying

CHECKED FROM IMAGE

Answer: A

53. Which of these helps loosen secretions in pneumonia?

- A. Dehydration
- B. Bed rest
- C. Warm fluid intake
- D. Cold air

CHECKED FROM IMAGE

Answer: C

54. A complication of pneumonia where fluid accumulates around the lungs is:

- A. Shock
- B. Pleural effusion
- C. Hypertension
- D. Obesity

CHECKED FROM IMAGE

Answer: B

55. Nursing management for pneumonia includes monitoring:

- A. Vital signs
- B. Eye sight
- C. Hearing
- D. Hair growth

CHECKED FROM IMAGE

Answer: A

56. Frequent suctioning can be a:

- A. Predisposing factor
- B. Cure
- C. Diagnostic test
- D. Vaccine

CHECKED FROM IMAGE

Answer: A

57. Antimicrobial therapy choice depends on:

- A. Patient's age
- B. Sputum culture
- C. Weight
- D. Gender

CHECKED FROM IMAGE

Answer: B

58. Preventive measures for pneumonia include:

- A. Smoking
- B. Immunization
- C. Immobility
- D. Shallow breathing

CHECKED FROM IMAGE

Answer: B

59. ABGs are followed to determine:

- A. Blood type
- B. Oxygen need
- C. Glucose level
- D. Bone density

CHECKED FROM IMAGE

Answer: B

60. Which drug relieves rhinorrhea in pneumonia?

- A. Antibiotics
- B. Antihistamines
- C. Aspirin
- D. Vitamin C

CHECKED FROM IMAGE

Answer: B

61. TB is primarily an infection of the:

- A. Liver
- B. Lungs
- C. Heart
- D. Skin

CHECKED FROM IMAGE

Answer: B

62. How is TB transmitted?

- A. Skin contact
- B. Aerosol droplets
- C. Contaminated water
- D. Blood transfusion

CHECKED FROM IMAGE

Answer: B

63. A cough suspicious of TB usually lasts more than:

- A. 1 week
- B. 2 weeks
- C. 3 weeks
- D. 1 day

CHECKED FROM IMAGE

Answer: C

64. Systemic symptoms of TB include:

- A. Weight gain
- B. High energy
- C. Night sweats
- D. Increased appetite

CHECKED FROM IMAGE

Answer: C

65. Extra-pulmonary TB can affect the:

- A. Central nervous system
- B. Hair
- C. Nails
- D. Teeth

CHECKED FROM IMAGE

Answer: A

66. Which antibiotic is commonly used to treat TB?

OPTIONS NEED REVIEW

Answer: B

67. To promote airway clearance in TB, the nurse should encourage:

OPTIONS NEED REVIEW

Answer: A

68. Pyridoxine (Vitamin B6) is given to TB patients to prevent:

OPTIONS NEED REVIEW

Answer: A

69. TB patients are taught to cover their mouth when:

OPTIONS NEED REVIEW

Answer: C

70. Which room type is required for a hospitalised active TB patient?

OPTIONS NEED REVIEW

Answer: B

71. TB can spread through the:

OPTIONS NEED REVIEW

Answer: B

72. A complete TB evaluation includes:

OPTIONS NEED REVIEW

Answer: B

73. TB prevention in children involves:

OPTIONS NEED REVIEW

Answer: B

74. Monitoring weight in TB patients is part of:

OPTIONS NEED REVIEW

Answer: B

75. A complication of TB is involvement of the:

OPTIONS NEED REVIEW

Answer: A

76. COPD is characterised by:

OPTIONS NEED REVIEW

Answer: B

77. A major occupational risk factor for COPD is:

OPTIONS NEED REVIEW

Answer: B

78. Which enzyme inhibitor deficiency causes lung injury in COPD?

OPTIONS NEED REVIEW

Answer: B

79. A physical sign of COPD is:

OPTIONS NEED REVIEW

Answer: B

80. Pursed-lip breathing helps to:

OPTIONS NEED REVIEW

Answer: A

81. Which medication decreases airway inflammation in COPD?

OPTIONS NEED REVIEW

Answer: B

82. Pulmonary rehabilitation involves:

OPTIONS NEED REVIEW

Answer: A

83. To clear secretions in COPD, the nurse uses:

OPTIONS NEED REVIEW

Answer: B

84. Nebulised saline inhalations help to:

OPTIONS NEED REVIEW

Answer: B

85. COPD patients should monitor for tachycardia when taking:

OPTIONS NEED REVIEW

Answer: B

86. A technique to manage fatigue in COPD is:

OPTIONS NEED REVIEW

Answer: A

87. A sign of emphysema is:

OPTIONS NEED REVIEW

Answer: B

88. COPD can lead to which skeletal issue?

OPTIONS NEED REVIEW

Answer: B

89. In COPD, wheezing is heard through a:

OPTIONS NEED REVIEW

Answer: B

90. Depression is a possible _____ of COPD.

OPTIONS NEED REVIEW

Answer: C

MATCHING

1. Pneumonia (bacterial) A. Mycobacterium tuberculosis C
 2. Pulmonary Tuberculosis B. Aspiration of gastric content A
 3. Fungal Pneumonia C. Streptococcus bacteria E
 4. Inhalation Pneumonia D. Long-term tobacco smoke exposure B
 5. COPD E. Candida or Aspergillus species D
 6. Rifampicin and Isoniazid A. Dilate airways and improve airflow in COPD D
 7. Bronchodilators B. Relieve rhinorrhoea and sneezing in pneumonia A
 8. Antihistaminics C. Protect high-risk persons from pneumonia B
 9. BCG Vaccine D. Kill Mycobacterium tuberculosis E
 10. Pneumococcal Vaccine E. Protect children against tuberculosis C
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ESSAY

91. Define Pneumonia and list its four classifications.
 92. List five predisposing factors for pneumonia.
 93. What are the preventive measures for pneumonia?
 94. Explain how Pulmonary Tuberculosis is transmitted.
 95. Identify five risk factors for developing Tuberculosis.
 96. What are the systemic symptoms and common diagnostic tests for TB?
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97. Define COPD and list its primary risk factors.

98. Describe the nursing interventions to improve airway clearance in a COPD

99. List five potential complications of COPD.

100. Write a nursing care plan for a COPD patient including diagnoses, goals, and

Chapter 3 - Digestive System Disorders

TRUE / FALSE

1. Peptic ulcers can form in the esophagus.

Answer: T

2. Gastric ulcers are most commonly found in the greater curvature of the stomach.

Answer: F

3. H. pylori infection plays a role in the development of gastric ulcers.

Answer: T

4. Duodenal ulcers are often associated with protein-rich meals.

Answer: T

5. Stress ulcers usually occur after a chronic illness rather than an acute crisis.

Answer: F

6. In gastric ulcers, acid secretion is typically hyper-secretion.

Answer: F

7. Pain from a duodenal ulcer usually occurs 2–4 hours after meals.

Answer: T

8. Food typically aggravates the pain in a gastric ulcer.

Answer: T

9. Vomiting is a common symptom of duodenal ulcers.

Answer: F

10. Duodenal ulcers are more likely to manifest as haematemesis than gastric ulcers.

Answer: F

11. Melaena is more common with duodenal ulcers.

Answer: T

12. Fibre optic endoscopes allow direct visualisation of the stomach and duodenum.

Answer: T

13. Patients should limit fluids for 48 hours after a barium X-ray.

Answer: F

14. Barium may cause the stool to appear white for several days.

Answer: T

15. Haemorrhage is a potential complication of peptic ulcers.

Answer: T

16. Orthostatic hypotension is a manifestation of ulcer haemorrhage.

Answer: T

17. "Coffee ground" vomitus is more likely associated with a gastric ulcer.

Answer: T

18. A saline lavage via NGT is used during the management of haemorrhage.

Answer: T

19. Perforation is when GI contents empty into the peritoneal cavity.

Answer: T

20. The pain from perforation is described as dull and localised.

Answer: F

21. A rigid, tender abdomen is a sign of perforation.

Answer: T

22. Perforation is considered a surgical emergency.

Answer: T

23. Pyloric obstruction is caused by excessive acid only.

Answer: F

24. Persistent vomiting is a symptom of pyloric obstruction.

Answer: T

25. Pyloric obstruction can lead to metabolic acidosis.

Answer: F

26. Conservative therapy for ulcers includes the elimination of smoking.

Answer: T

27. Antibiotics are used to eradicate H. pylori infections.

Answer: T

28. Recurrence of ulcers is much higher (75–90%) if H. pylori is not treated.

Answer: T

29. Antacids are the initial drugs of choice for medical management.

Answer: T

30. Histamine H2 receptor antagonists increase HCl secretion.

Answer: F

31. Anticholinergic drugs speed up gastric motility.

Answer: F

32. Gastroduodenostomy involves connecting the stomach to the jejunum.

Answer: F

33. Gastrojejunostomy is the removal of the lower stomach and connecting it to the jejunum.

Answer: T

34. Total gastrectomy involves the removal of the entire stomach.

Answer: T

35. In a total gastrectomy, the esophagus is connected directly to the jejunum.

Answer: T

36. Subtotal gastrectomy involves removing the distal third of the stomach.

Answer: T

37. Hypokalaemia is a risk in patients with pyloric obstruction.

Answer: T

38. The "back diffusion" of HCl damages the gastric epithelium.

Answer: T

39. Bile reflux from the duodenum cannot damage the gastric mucosal barrier.

Answer: F

40. Duodenal ulcers lead to a large acid load in the duodenum.

Answer: T

41. Ischaemia is a contributing factor to the evolution of stress ulcers.

Answer: T

42. Upper GI series uses barium to detect structural disorders.

Answer: T

43. NGT suction helps prevent distension during haemorrhage management.

Answer: T

44. Perforation management includes keeping the patient NPO (nothing by mouth).

Answer: T

45. Stomach emptying is enhanced in pyloric obstruction.

Answer: F

MCQ

46. Which of the following is NOT a location for a peptic ulcer?

OPTIONS NEED REVIEW

Answer: D

47. Gastric ulcers are most common near the:

OPTIONS NEED REVIEW

Answer: B

48. Which bacteria is heavily linked to peptic ulcers?

OPTIONS NEED REVIEW

Answer: C

49. What is the primary cause of duodenal ulcers?

OPTIONS NEED REVIEW

Answer: A

50. Stress ulcers are most common in which parts of the GI tract?

OPTIONS NEED REVIEW

Answer: B

51. Pain that occurs 1–2 hours after a meal is characteristic of:

OPTIONS NEED REVIEW

Answer: A

52. Which ulcer type is often relieved by food?

OPTIONS NEED REVIEW

Answer: B

53. Haematemesis is more likely to occur with which type of ulcer?

OPTIONS NEED REVIEW

Answer: A

54. Melaena refers to:

OPTIONS NEED REVIEW

Answer: B

55. The procedure used to visualise the esophagus, stomach, and duodenum directly is:

OPTIONS NEED REVIEW

Answer: B

56. Following an Upper GI series, why is a mild laxative sometimes recommended?

OPTIONS NEED REVIEW

Answer: B

57. A "coffee ground" appearance of vomitus indicates:

OPTIONS NEED REVIEW

Answer: B

58. Signs of orthostatic hypotension during haemorrhage include:

OPTIONS NEED REVIEW

Answer: B

59. Management of haemorrhage includes all the following EXCEPT:

OPTIONS NEED REVIEW

Answer: C

60. Perforation leads to the emptying of GI contents into the:

OPTIONS NEED REVIEW

Answer: B

61. A patient with a "rigid, board-like" abdomen likely has:

OPTIONS NEED REVIEW

Answer: C

62. Which is a medical emergency requiring urgent surgery?

OPTIONS NEED REVIEW

Answer: C

63. Pyloric obstruction is caused by:

OPTIONS NEED REVIEW

Answer: B

64. Metabolic alkalosis in pyloric obstruction is caused by:

OPTIONS NEED REVIEW

Answer: D

65. Conservative therapy for ulcers includes:

OPTIONS NEED REVIEW

Answer: B

66. The primary goal of using antibiotics in ulcer treatment is:

OPTIONS NEED REVIEW

Answer: B

67. Initial drugs of choice for medical management are:

OPTIONS NEED REVIEW

Answer: B

68. Which medication type stops cholinergic stimulation of HCl?

OPTIONS NEED REVIEW

Answer: B

69. Gastroduodenostomy connects the remaining stomach to the:

OPTIONS NEED REVIEW

Answer: C

70. Subtotal gastrectomy removes how much of the stomach?

OPTIONS NEED REVIEW

Answer: B

71. Which surgical procedure connects the esophagus to the jejunum?

OPTIONS NEED REVIEW

Answer: C

72. "Back diffusion" refers to HCl damaging the:

OPTIONS NEED REVIEW

Answer: B

73. Protein-rich meals contribute to duodenal ulcers because they:

OPTIONS NEED REVIEW

Answer: A

74. Which ulcer type is most associated with rapid emptying of food?

OPTIONS NEED REVIEW

Answer: B

75. Ischaemia is a major factor in:

OPTIONS NEED REVIEW

Answer: C

76. In an Upper GI series, the patient ingests:

OPTIONS NEED REVIEW

Answer: C

77. Clinical manifestations of haemorrhage include:

OPTIONS NEED REVIEW

Answer: B

78. The purpose of NGT suction in haemorrhage is to:

OPTIONS NEED REVIEW

Answer: C

79. Perforation pain can spread to:

OPTIONS NEED REVIEW

Answer: C

80. Which is NOT a management step for perforation?

OPTIONS NEED REVIEW

Answer: C

81. Pyloric obstruction is a symptom of:

OPTIONS NEED REVIEW

Answer: B

82. Abdominal bloating is a symptom of:

OPTIONS NEED REVIEW

Answer: C

83. Histamine H2 receptor antagonists work by:

OPTIONS NEED REVIEW

Answer: B

84. Gastrojejunostomy is also known as a procedure that reconnects the:

OPTIONS NEED REVIEW

Answer: C

85. Dietary modifications are part of:

OPTIONS NEED REVIEW

Answer: A

86. The recurrence rate of ulcers with H. pylori Rx is:

OPTIONS NEED REVIEW

Answer: A

87. Total gastrectomy is the:

OPTIONS NEED REVIEW

Answer: B

88. Management of haemorrhage requires replacing:

OPTIONS NEED REVIEW

Answer: C

89. Bright red haematemesis is more likely in:

OPTIONS NEED REVIEW

Answer: A

90. Peptic ulcers are essentially:

OPTIONS NEED REVIEW

Answer: B

MATCHING

1. Gastric Ulcer A. Manifests as rigid, tender abdomen and sudden sharp pain C

2. Duodenal Ulcer B. Involves reconnecting stomach to jejunum after removing part D

3. Perforation C. Characterised by pain 1–2 hours after meals; vomiting common A

4. Pyloric Obstruction D. Pain often relieved by food; may be relieved by food E

5. Gastrojejunostomy E. Leads to abdominal bloating, persistent vomiting, hypokalaemia B

6. Stress Ulcer C. Occurs after acute medical crisis, trauma, or major surgery C

7. Upper GI Series E. Diagnostic test involving barium ingestion; may cause white stools E

8. Haematemesis D. Bloody or coffee-ground vomit — more common with gastric ulcers D

9. H. pylori C. Bacterium — major cause of ulcer recurrence C

10. H2 Receptor Antagonists D. Block final intracellular activator of HCl secretion D

ESSAY

91. Define peptic ulcer and list the three main types.

92. Compare the timing and nature of pain between Gastric and Duodenal ulcers.

93. Explain the role of H. pylori in the development of peptic ulcers.

94. What is the "Upper GI series" and what are the nursing responsibilities after

95. Describe the clinical manifestations of ulcer perforation.

96. Outline the management steps for a patient experiencing a GI haemorrhage.

97. What causes pyloric obstruction and what are its symptoms?

98. Compare conservative and surgical management of peptic ulcers.

99. Describe the types of surgical procedures used for peptic ulcers.

100. Write a nursing care plan for a patient with peptic ulcer including diagnoses,

Chapter 4 - Cardiovascular Disorders

TRUE / FALSE

1. Angina is defined as irreversible necrosis of heart muscle secondary to prolonged ischaemia.

Answer: F

2. Physical exertion can precipitate an angina attack by increasing myocardial oxygen demands.

Answer: T

3. Angina pain typically radiates to the neck, jaw, shoulders, and inner aspects of the upper extremities.

Answer: T

4. Classic angina is characterised by increasing frequency and duration of symptoms over time.

Answer: F

5. Nocturnal angina occurs only at night and is always related to a recumbent position.

Answer: F

6. Angina decubitus is discomfort precipitated by recumbent positions and relieved by sitting or standing.

Answer: T

7. During an acute angina attack, the patient's vital signs are always normal.

Answer: F

8. An exercise stress test involves performing an ECG while the patient exercises on a treadmill or stationary bicycle.

Answer: T

9. Chest X-ray findings are typically abnormal in patients with stable angina.

Answer: F

10. Nitroglycerin tablets should be stored in a light-coloured plastic bottle.

Answer: F

11. Cotton should be placed in the nitroglycerin bottle to protect the tablets.

Answer: F

12. If angina pain is not relieved after the first nitroglycerin tablet, a second tablet can be taken 3 to 5 minutes later.

Answer: T

13. Patients with angina should swallow their saliva while waiting for a sublingual nitroglycerin tablet to dissolve.

Answer: F

14. Headache is a known side effect of nitroglycerin.

Answer: T

15. Patients with angina should exercise vigorously immediately after meals.

Answer: F

16. Acute Myocardial Infarction (MI) is defined as irreversible necrosis of heart muscle secondary to prolonged ischaemia.

Answer: T

17. Hyperlipidaemia is one of the six primary risk factors for atherosclerotic coronary artery disease and MI.

Answer: T

18. Diaphoresis and dyspnea are common clinical manifestations of acute MI.

Answer: T

19. In acute MI, serum CPK levels typically decrease.

Answer: F

20. The initial ECG evaluation in an MI patient should be completed within 10 minutes of arrival.

Answer: T

21. Aspirin 160–325 mg chewed is part of initial emergency management of acute MI.

Answer: T

22. Sublingual TNG is contraindicated in MI when systolic blood pressure is below 90 mmHg.

Answer: T

23. Rehabilitation after MI aims to keep the patient as inactive as possible.

Answer: F

24. One sub-goal of cardiac rehabilitation is to help the client accept the limitations imposed by illness.

Answer: T

25. Occupational adjustment is NOT considered part of cardiac rehabilitation goals.

Answer: F

26. Eating heavy meals can precipitate angina by increasing demands on the heart.

Answer: T

27. Stress and emotionally provoking situations can precipitate angina by releasing adrenaline.

Answer: T

28. Exposure to cold is NOT considered a precipitating factor for angina.

Answer: F

29. Unstable angina shows a progressive pattern of increasing frequency, intensity, and duration of symptoms.

Answer: T

30. The nursing diagnosis 'anxiety related to fear of death' is appropriate for a patient with angina.

Answer: T

MCQ

31. Angina is best described as:

- A. Irreversible necrosis of the myocardium
- B. A clinical syndrome characterised by paroxysmal chest pain or pressure
- C. Inflammation of the pericardium

Answer: B

32. Which of the following is a precipitating factor for angina?

OPTIONS NEED REVIEW

Answer: B

33. The type of angina that occurs with recumbent position and is relieved by sitting is:

OPTIONS NEED REVIEW

Answer: D

34. Which type of angina is described as preinfarction angina?

OPTIONS NEED REVIEW

Answer: B

35. During an acute angina attack, the nurse would expect the patient's heart rhythm to be:

OPTIONS NEED REVIEW

Answer: C

36. Which diagnostic test typically shows NORMAL results in a patient with stable angina?

OPTIONS NEED REVIEW

Answer: C

37. The pharmacological treatment of angina includes all of the following EXCEPT:

OPTIONS NEED REVIEW

Answer: D

38. The surgical revascularisation procedure for angina is:

OPTIONS NEED REVIEW

Answer: B

39. Which of the following is the FIRST action when a patient experiences angina chest pain?

OPTIONS NEED REVIEW

Answer: B

40. Nitroglycerin tablets must be stored in:

OPTIONS NEED REVIEW

Answer: B

41. If angina pain persists after the first nitroglycerin tablet, the patient should:

OPTIONS NEED REVIEW

Answer: B

42. Common side effects of nitroglycerin include all of the following EXCEPT:

OPTIONS NEED REVIEW

Answer: D

43. Patients with angina are advised to avoid physical exercise for how long after meals?

OPTIONS NEED REVIEW

Answer: C

44. Which dietary recommendation is appropriate for a patient with angina?

OPTIONS NEED REVIEW

Answer: C

45. A patient with angina is going outside in cold weather. Which advice is MOST appropriate?

OPTIONS NEED REVIEW

Answer: B

46. Acute Myocardial Infarction (MI) is caused by:

OPTIONS NEED REVIEW

Answer: C

47. Which of the following is NOT one of the six primary risk factors for MI?

OPTIONS NEED REVIEW

Answer: C

48. Which ECG changes are associated with acute MI?

OPTIONS NEED REVIEW

Answer: B

49. Which laboratory finding is expected in acute MI?

OPTIONS NEED REVIEW

Answer: B

50. The initial emergency ECG in a suspected MI patient should be performed within:

OPTIONS NEED REVIEW

Answer: C

51. Sublingual nitroglycerin is CONTRAINDICATED in MI when systolic blood pressure is:

OPTIONS NEED REVIEW

Answer: C

52. Which analgesic is mentioned as part of the initial emergency management of acute MI?

OPTIONS NEED REVIEW

Answer: C

53. The dose of aspirin recommended in the initial management of acute MI is:

OPTIONS NEED REVIEW

Answer: C

54. Which of the following is a clinical manifestation of acute MI?

OPTIONS NEED REVIEW

Answer: B

55. Syncope in the context of acute MI is described as:

OPTIONS NEED REVIEW

Answer: C

56. One of the main goals of cardiac rehabilitation is:

OPTIONS NEED REVIEW

Answer: B

57. How many sub-goals of rehabilitation are specifically listed in the lecture?

OPTIONS NEED REVIEW

Answer: D

58. Which nursing diagnosis is appropriate for a patient experiencing chest pain due to myocardial ischaemia?

OPTIONS NEED REVIEW

Answer: B

59. The nursing intervention of health teaching for angina patients should include:

OPTIONS NEED REVIEW

Answer: B

60. Which of the following is an objective indicator of angina?

OPTIONS NEED REVIEW

Answer: B

61. The psychosocial nursing diagnosis for angina patients commonly includes:

OPTIONS NEED REVIEW

Answer: B

62. A patient with angina reports pain only at night. This is best classified as:

OPTIONS NEED REVIEW

Answer: C

63. Which dietary substance should be restricted in angina patients because it increases heart rate?

OPTIONS NEED REVIEW

Answer: B

64. During a nursing assessment for angina, which question is MOST important to ask?

OPTIONS NEED REVIEW

Answer: B

65. The nitroglycerin tablet for angina should be placed:

OPTIONS NEED REVIEW

Answer: B

66. ECG changes during an acute angina attack may show:

OPTIONS NEED REVIEW

Answer: D

67. Which of the following best describes the pain of angina?

OPTIONS NEED REVIEW

Answer: B

68. Coronary angiography is used to:

OPTIONS NEED REVIEW

Answer: B

69. A patient with angina should be taught to:

OPTIONS NEED REVIEW

Answer: B

70. Which of the following is the correct statement about nitroglycerin prophylactic use?

OPTIONS NEED REVIEW

Answer: B

71. Which of the following is a risk factor for angina listed in the lecture?

OPTIONS NEED REVIEW

Answer: D

72. Which of the following is a risk factor for angina listed in the lecture?

OPTIONS NEED REVIEW

Answer: C

73. Which statement about Type A personality and angina is correct?

OPTIONS NEED REVIEW

Answer: B

74. The patient teaching about Type A personality in angina should emphasise:

OPTIONS NEED REVIEW

Answer: B

75. A complication of TB is involvement of the:

OPTIONS NEED REVIEW

Answer: C

76. In CRF management, which associated diseases must be controlled first?

OPTIONS NEED REVIEW

Answer: B

77. Which imaging technique is mentioned as a diagnostic test for acute MI?

OPTIONS NEED REVIEW

Answer: B

78. Beta blockers are used in angina management because they:

OPTIONS NEED REVIEW

Answer: B

79. Which test is used to evaluate cardiac function under physical stress?

OPTIONS NEED REVIEW

Answer: C

80. Which of the following statements about obesity and angina is correct?

OPTIONS NEED REVIEW

Answer: B

81. Which of the following statements about obesity and angina is correct?

OPTIONS NEED REVIEW

Answer: B

82. What is the goal of reducing anxiety in a patient with angina?

OPTIONS NEED REVIEW

Answer: B

83. The primary nursing assessment question to the nurse is the one listed in the lecture?

OPTIONS NEED REVIEW

Answer: B

84. Psychosocial factors in cardiac rehabilitation include:

OPTIONS NEED REVIEW

Answer: B

85. Risk factor modification for angina includes all of the following EXCEPT:

OPTIONS NEED REVIEW

Answer: C

86. In acute MI management, oxygen is administered via:

OPTIONS NEED REVIEW

Answer: B

87. Which of the following is a clinical manifestation of angina that represents the MOST severe form?

OPTIONS NEED REVIEW

Answer: B

88. The term 'preinfarction angina' refers to:

OPTIONS NEED REVIEW

Answer: D

89. Which of the following conditions is associated with an increased risk of UTI due to urinary stasis?

OPTIONS NEED REVIEW

Answer: B

90. Which of the following is the correct statement about nitroglycerin prophylactic use?

OPTIONS NEED REVIEW

Answer: B

91. Which of the following is the MOST appropriate advice for a patient regarding smoking?

OPTIONS NEED REVIEW

Answer: C

92. Which of the following correctly describes pressure in the lower pelvis as a symptom?

OPTIONS NEED REVIEW

Answer: C

MATCHING

1. Classic Angina A. Irreversible necrosis of heart muscle — prolonged ischaemia D

2. Unstable Angina B. Angina occurring only at night E

3. Nocturnal Angina C. Discomfort precipitated by recumbent positions B

4. Angina Decubitus D. Short duration pain by exertion, relieved by rest C

5. Myocardial Infarction E. Progressive pattern of increasing intensity and duration A

6. Relief of Angina pain A. Usually severe and may be silent B

7. Severity of MI pain B. Relieved by rest and nitrates A

8. Common MI symptoms C. Physical exertion, cold exposure, or heavy meals E

9. Angina precipitating factors D. Increase in serum CPK C

10. MI Diagnostic Lab Study E. Nausea and vomiting D

11. Nitroglycerin storage Keep in dark-coloured glass bottle Keep in dark-coloured glass bottle

12. Sublingual NTG use Place under tongue; do not swallow saliva until dissolved Under tongue; do not swallow saliva

13. Diet education for Angina Low fat, small frequent meals; avoid caffeine Low-fat diet, small frequent meals, avoid caffeine

14. Cold weather precaution Cover mouth and nose with a scarf Cover mouth and nose with scarf

15. Initial MI emergency care Obtain ECG in less than 10 minutes ECG in less than 10 minutes

ESSAY

93. Define angina pectoris and describe its four main types with their

94. Describe the nursing management of a patient with angina, including

95. Explain the health education that should be provided to a patient with angina

96. Outline the initial emergency management steps for a patient presenting with

97. Describe the six sub-goals of cardiac rehabilitation after myocardial infarction

98. List and describe the six primary risk factors for myocardial infarction.

99. Compare and contrast angina pectoris and myocardial infarction in terms of

100. Write a comprehensive nursing care plan for a patient with acute myocardial
